



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : زيزى محمد بيومى عطيه
المحافظة : الشرقيه
الحى / المدينة : الزقازيق
رقم البطاقة : 26502041300101 رقم التليفون : 01065705007
العمر (تاريخ الميلاد) : 04-02-1965 المهنة : ربه منزل
العنوان : ///
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: زيزى محمد بيومى عطيه National ID: 26502041300101	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



09/07/2026

MDT FORM

Name:	ZIZY Mohamed Byounny Atiah	MRN:	4287
Age:	62 Y	ID:	26502041300101
Menopausal Sta:	post	Phone:	01283570671
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	htn	Reason Of MDT:	mbc

Clinical Data:

BX: IDC (P52)
ER: 7/8 PR: 5/8 HER2: +3 KI67:

R

L

Sonomammography:

Mets Workup:

2/2023 denovo metastatic breast cancer to liver and lung
1st line chemo Paclitaxel + Trastutumab IV
received 31 wks. of taxol+11 cycles of Herceptin

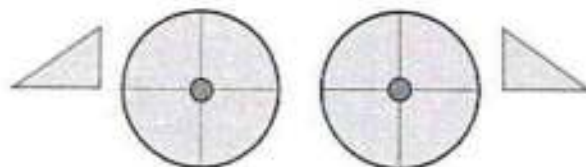
CT cap:SD

PATIENT SHOWED SD IN LUNGS, NEAR CR IN LIVER, PR IM LUNGS AND AXILLA
kept on maintenance Herceptin + AI+ zometa till 11/2024

11/2024 CT CAP: DP of HFLS

12/2025 mdt: 2nd line XELODA-TYKERB

7/2026 CT CAP: Progressive course of the pulmonary metastases
• NO HFL II



T N M

History:

Recommendations:

MDT Outcome & Plan

For trial of TDM1 through NGO.....Meanwhile, shift to Gemzar/Carboplatin and
maintain on Tykerb.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هاشم عبدالمجيد	د. هاشم عبدالمجيد				

2/5/2026
7/11/2026

Patient's Name:

Zizi

Mohammed Beyoumi Atieh



Date:

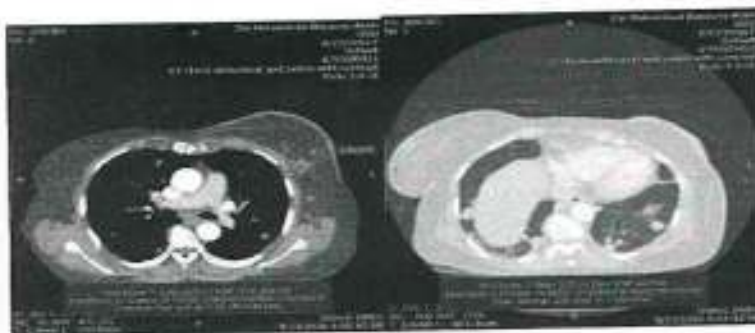
2026-08-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUN OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Compare to previous study dated 15/4/2026, the current study showed
- An enhancing neoplastic mass lesion is seen in the deep upper outer quadrant of the left breast with central area of breaking down and /or cystic degeneration and measures about 24x21 mm in dimensions. Another spiculated lesion is also seen in the supra-areolar region of the same breast with foci of microcalcifications noted inside and measures about 36x26mm in dimensions .The overlying skin is thickened, and measures about 10 mm.
- Pathologically-enlarged left axillary lymph nodes are seen with loss of their hilar fat contents and the largest one measures about 14x11mm in dimensions.
- Normal right breast.
- Benign-looking right axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 7mm.
- Hyperinflated both lungs.
- Enhancing pretracheal and carinal/subcarinal lymph nodes are seen, and each globally measures measuring about 36x11mm and 37x17mm in dimensions.
- Progressive course in size and numbers of the multiple variable sized enhancing bilateral pulmonary nodules are seen with the largest one seen in the anterior segment of right lower pulmonary lobe and measures about 17 mm.
- Linear atelectatic changes are seen in the lingula and in both lower pulmonary lobes.
- No other patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions seen.
- Average size of the heart with normal CT of its big vessels.
- Osteolytic bone lesion is still seen in the body of the sternum.

Patient's Name:

Zizi

Mohammed Beyoumi Atieh



Date:

2026-08-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- Average sized liver, measuring about 14.5 cm, showing a normal parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen, measuring about 9.5 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is seen and measures about 18x18mm in dimensions.
- The rest of the pelvic structures are unremarkable..

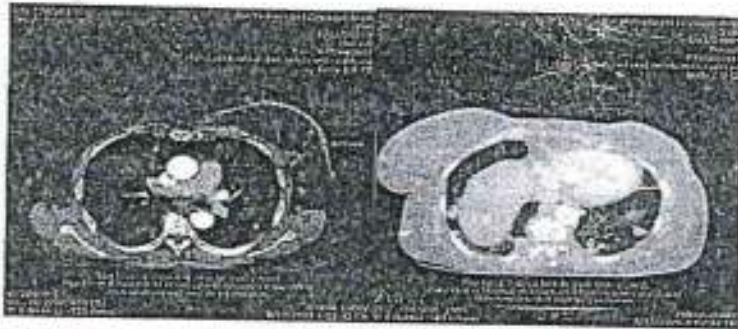
IMPRESSIONS:

- **Progressive course of the pulmonary metastases**
- **Stationary rest of study**

Prof/Hanan El-Ahwal.

Patient's Name: Zizi Mohammed Beyoumi Atieh
Date: 2026-08-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

د. مكا
[Signature]



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Patient Name : الأستاذ / زيزي محمد بيومي عطيه	Request No. : 13006458
Patient No. : 26502041200101	Request Date : 14/12/2022
Path No. : SR -973/22	Verification Date : 18/12/2022
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

CLINICAL DATA : Right breast lesion

NATURE OF SPECIMEN

Revision of tru cut biopsy & IHC slides

GROSS EXAMINATION

One block + 5 slides .

MICROSCOPIC EXAMINATION

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis) : (3+2+2) Grade II
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8)
- * PR: Strong nuclear reaction in 12% of tumor cells (Allred score 5/8)
- * HER2: REPEATED
- * SISH: Amplified with cluster

DIAGNOSIS

Left breast mass ; slide revision of tru cut biopsy specimen :

Invasive duct carcinoma, Grade II .

- * ER: Positive (score 7/8).
- * PR: Positive (score 5/8).
- * HER2: REPEATED

Prof. Dr. Lubna Omer

Verified by :
saamah sayed

Printing Date : 24/10/2024 09:17:49

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - M^{rs} El Qadima - Cairo, Egypt



Patient Name	: الأستاذة / زيزي محمد بيومي عطيه	Request No.	: 13006458
Patient No.	: 26502041200101	Request Date	: 14/12/2022
Path No.	: SR -973/22	Verification Date	: 25/12/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة النولة
Age	: 57 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast lesion.
Nature of Specimen: Slide revision.

Gross Examination:

- One block + 5 slides.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for HER-2 clone (4B5).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8)
- * PR: Strong nuclear reaction in 12% of tumor cells (Allred score 5/8)
- * HER2: Strong complete membrane reaction in >10% of tumor cells (score 3+)
- * SISH: Amplified with cluster

Diagnosis:

Left breast mass, slide revision:
Invasive duct carcinoma, Grade II.

- * ER: Positive (score 7/8).
- * PR: Positive (score 5/8).
- * HER2: Positive (score 3+).

Prof. Dr. Lubna Omer

Verified by :
Dr.Amin Ahmed

Printing Date : 22/08/2024 12:08:07

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	الأستاذة / زيزي محمد بيومي عطية	Request No.	13006458
Patient No.	26502041200101	Request Date	14/12/2022
Path No.	SR -973/22	Verification Date	25/12/2022
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	57 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Verified by :
Dr.Amin Ahmed

Printing Date : 22/08/2024 12:08:07

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Zizi Mohamed Bayoumi Atteya	Date : 27/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 60 Year(s)
Medical ID: 4868/2022	Pat. ID: 31977

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

- Compared to the previous study dated 16-10-2024.

I Chest:

- Left breast shows stationary few parenchymal nodular densities and skin thickening & prominent left axillary lymph nodes are still noted.
- **Regressive course regarding** some previously noted few bilateral pulmonary nodules and the largest at lingula still measures 11 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- average cardiac size No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and great vessels

II Abdomen & pelvis:

- Enlarged liver showing regressive **course** of size of previously noted segment IVb/V hypodense focal lesions the largest measures currently about 2.3x2.7 cm
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Still noted uterine fundus small partially calcified lesion (fibroid).
- Stable sclerotic bone lesion in sternal body.
- Dorsal & lumbar vertebral spondylotic changes.

Opinion:

Status follow up showing:

- **Regressive course of previously noted some pulmonary nodules and hepatic focal lesions**
- **Rather stationary rest of the study.**

Dr. Ahmed Elgazar

Prof. Menna Talla Hanafy, M.D.

Name: Mrs. ZIZI MOHAMED BAYOUMI

Date: 2026-09-02

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven left breast cancer, post NACT assessment study with no available radiological data for comparison .

Findings:

Left breast.

- Scattered densities of glandular tissue (ACR B)
- An irregular shaped central dense mass with pleomorphic microcalcifications is seen ..
- Thickened skin.
- Retracted nipple .
- Ultrasound shows the pathologically proved malignant mass is seen infiltrating ,most of the inner half of the breast measuring (6 x 4.5 x 4 cm).
- Thickened skin reaching 8 mm with coarsened trabeculae and minimal interstitial as well as subdermal edema.
- Two small axillary lesions with calcifications ,the largest measures (15 x 14 mm) ...mostly pathological nodal with extranodal affection .. No pathological infra or supra-clavicular lymph nodes.

Right breast.

- Scattered densities of glandular tissue (ACR B)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Left breast (BIRADS 6): Locally advanced malignant mass .

Right breast (BIRADS 1): Normal

Best Regards,

DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Handwritten notes: 2/15/2026, 2/14/2026, 2/13/2026

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
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SIGNATURE

MDT Outcome & Plan

Recommendations:

History:

7/2026 CT CAP: • Progressive course of the pulmonary metastases
• NO HFL II

12/2025 mdt: 2nd line XELODA-TYKERB

11/2024 CT CAP: DP of HFLS

T N M

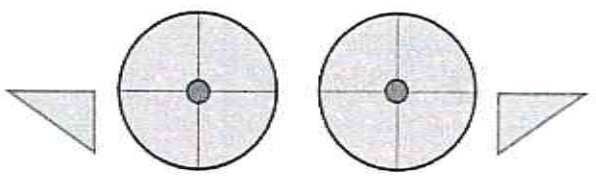
PATIENT SHOWED SD IN LUNGS, NEAR CR IN LIVER, PR IN LUNGS AND AXILLA

CT cap: SD

2/2023 denovo metastatic breast cancer to liver and lung

1st line chemo Paclitaxel + Trastuzumab IV

received 31 wks. of taxol+11 cycles of Herceptin



Met Workup:

Sonomammography:

ER: 7/8 PR: 5/8 HER2: +3 KI67: IDC (P52)

BX: IDC (P52)

Clinical Data:

Name:	ZIZY Mohamed Byoumy Atiah
MRN:	4287
ID:	26502041300101
Menopausal Sta post	
NRS SCORE:	
Family History:	
Marital Status:	
Phone:	01283570671
Reason of MDT:	mbc

MDT FORM

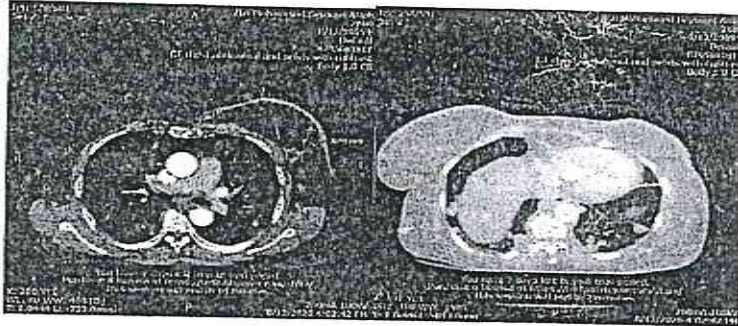
09/07/2026

Logo of the Ministry of Health, Kingdom of Saudi Arabia

Logo of the Ministry of Health, Kingdom of Saudi Arabia

Patient's Name: Zizi Mohammed Beyoumi Atieh
Date: 2026-08-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

دكتور
عبد
المنعم

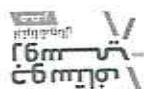


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- Normal right breast.
- Benign-looking right axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 7mm.
- Hyperinflated both lungs.
- Enhancing pretracheal and carinal/subcarinal lymph nodes are seen, and each globally measures measuring about 36x11mm and 37x17mm in dimensions.
- Multiple variable sized enhancing bilateral pulmonary nodules are seen with the largest one seen in the anterior segment of right lower pulmonary lobe and measures about 17 mm.
- Linear atelectatic changes are seen in the lingula and in both lower pulmonary lobes.
- No other patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions seen.
- Average size of the heart with normal CT of its big vessels.
- Osteolytic bone lesion is still seen in the body of the sternum.
- Average sized liver, measuring about 14.5 cm, showing a normal parenchymal attenuation and a homogenous enhancement pattern.
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- No sizable abdominal or pelvic lymphadenopathy.

أحمد

Patient's Name: Zizi Mohammed Beyoumi Atieh



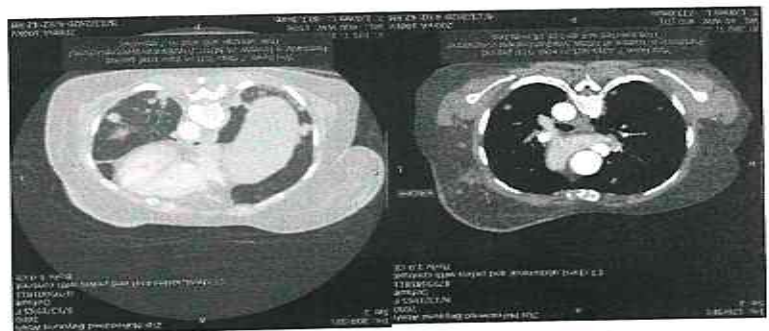
2026-08-13

Prof. Dr. no doctor

Goustave Roussey International Egypt

Referred by: Hospital:

Date:



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Date:

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IMPRESSIONS:

- Progressive course of the pulmonary metastases
- Stationary rest of study

Prof/Hanan El-Ahwal.

Name: Mrs. ZIZI MOHAMED BAYUMI

Date: 2026-09-02

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revealed:

Clinical summary: Pathologically proven left breast cancer, post NACT assessment study with no available radiological data for comparison.

Findings:

Left breast.

- Scattered densities of glandular tissue (ACR B)
- An irregular shaped central dense mass with pleomorphic microcalcifications is seen ..
- Thickened skin.
- Retracted nipple .
- Ultrasound shows the pathologically proved malignant mass is seen infiltrating , most of the inner half of the breast measuring (6 x 4.5 x 4 cm).
- Thickened skin reaching 8 mm with coarsened trabeculae and minimal interstitial as well as subdermal edema.
- Two small axillary lesions with calcifications , the largest measures (15 x 14 mm) ..mostly pathological nodal with extranodal affection .. No pathological infra or supra-clavicular lymph nodes.

Right breast.

- Scattered densities of glandular tissue (ACR B)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Left breast (BIRADS 6): Locally advanced malignant mass .
Right breast (BIRADS 1): Normal

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

Suspected Cancer Risk

0 %	BIRADS 0: Non-informative study.
0 %	BIRADS 1: Normal study, routine screening
0 %	BIRADS 2: Benign findings, routine screening
2 %	BIRADS 3: Probably benign, for close follow up.
50 %	BIRADS 4: Probably malignant, for biopsy.
	A: mild suspicion.
	B: moderate suspicion.
	C: high suspicion.
98-100%	BIRADS 5: Malignant, for biopsy.
	BIRADS 6: Pathologically proven malignant

PATIENT NAME Zeze MOHAMMED BAYYOMI ATEA	PATIENT ID #: 2022210311
GENDER: Female	ORDER #: 1989097,1989098
AGE: 57 y	DATE OF SERVICE: 16-Nov-2022 03:41 PM

Clinical indication - all studies

Report: - all studies

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

REPORT:

- Scattered fibroglandular densities (ACR b).
- No speculated mass lesions or microcalcific clusters.
- Left breast UOQ pleomorphic calcifications are seen measuring about 3.1 x 4.2 cm corresponding on ultrasound to a para-areolar 2 o'clock ill defined lesion measuring about 3 x 2 cm with smaller nodules are seen surrounding it.
- Diffuse left breast skin thickening is seen measuring about 7 mm.
- Normal nipple areola complexes.
- Nonspecific right axillary lymphadenopathy.
- Left axillary pathologically enlarged left axillary lymph nodes are seen the largest measuring about 2.2 x 1.1 cm.

OPINION:

- *Left breast malignant looking masses as described above with pathologically enlarged left axillary lymph nodes (BIRADS 5).*

Recommendation - all studies

Referral to clinic

Reported By,

Dr. Ahmed Rozeka, MSc

16-Nov-2022 04:11 PM

PATIENT: Zeze MOHAMMED BAYYOMI ATEA, 2022210311, Female, 57 y
Report version 1, 07-Dec-2022 01:37 PM

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Patient Name: Zizi Mohamed Bayoumi	Date : 18/09/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 58 Year(s)
Medical ID: 4868/2022	Pat. ID: 31977

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

=

Compared to the previous study dated 22/06/2023.

I Chest: -

- Still noted left breast skin thickening with mild regression of the size of the enlarged left axillary pathological lymph nodes, the largest measuring about 1.1 x 1.7 cm.
- Stationary status of the bilateral few pulmonary nodules, the largest at lingula measuring 11 mm.
- A port a cath is still seen in place.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Enlarged liver showing homogenous texture with no definite hepatic focal lesions or area of abnormal enhancement.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Still noted sclerotic bone lesion in sternal body.
- Dorsal and lumbar vertebral spondylotic changes with osteophytic lipping.

Opinion:

Status follow up showing overall stationary course since the last study.

Dr. Fifi Samy Mahmoud, MSc

Dr. Ola Magdy, M.D.